Study of Intravitreal Aflibercept, Bevacizumab and Ranibizumab for Diabetic Macular Edema (T)

Ocular Exam Form (Randomization)

tblTOcularExamBaseline

PtID: **(tblPtRoster.PtID)**

Patient Initials:

Perform an ocular exam on both eyes, including examination of the anterior segment and posterior segment. Ocular examination must be done within 21 days prior to randomization.

To be eligible, the study eye can NOT have any of the following:

- **Macular edema considered to be due to a cause other than diabetic macular edema.**
- **An ocular condition such that, in the opinion of the investigator, visual acuity loss would not improve from resolution of macular edema (e.g., foveal atrophy, pigment abnormalities, dense subfoveal hard exudates, nonretinal condition).**
- **An ocular condition (other than diabetes) that, in the opinion of the investigator, might affect macular edema or alter visual acuity during the course of the study (e.g., vein occlusion, uveitis or other ocular inflammatory disease, neovascular glaucoma, etc.).**
- **Substantial cataract that, in the opinion of the investigator, is likely to be decreasing visual acuity by 3 lines or more (i.e., cataract would be reducing acuity to 20/40 or worse if eye was otherwise normal).**
- **Exam evidence of external ocular infection, including conjunctivitis, chalazion, or significant blepharitis**
- **Anticipated need for PRP within 6 months after randomization**
- **Anticipated need for major ocular surgery (including vitrectomy, cataract extraction, sclera buckle, or any intraocular surgery) within 6 months after randomization**

Non-Study Eye Eligibility:

- **If anti-VEGF treatment is indicated for any condition in the non-study eye at anytime during the study, the investigator must be willing to use the randomized anti-VEGF drug on the non-study eye.**

1. Ocular exam date: ___ ___/___ ___ ___/___ ___ ___ ___ **(OCTExamDtDaysFromEnroll)**
dd/MMM/yyyy

ANTERIOR SEGMENT EXAM – RIGHT EYE (OD)

Examination of lids/conjunctiva, cornea, iris, and anterior chamber

1. Anterior segment (other than lens): O Normal O Abnormal **(AntSegOthLensOD)**
If *Abnormal*, complete the following:

1a. Lids/Conjunctiva: O Normal O Abnormal **(LidsCondOD)**
If *Abnormal*, complete the following:

i. Describe any abnormalities _____

ii. Estimated effect on visual acuity? O None O 20/ 25- 20/40 O 20/50 – 20/100 O< 20/100 **(LidsEstVAOD)**

1b. Cornea (OD): O Normal O Abnormal **(CorneaCondOD)**
If *Abnormal*, complete the following:

i. Describe any abnormalities _____

ii. Estimated effect on visual acuity? O None O 20/25- 20/40 O 20/50 – 20/100 O< 20/100 **(CorneaEstVAOD)**

1c. Anterior chamber: O Normal O Abnormal **(AntChOD)**
If *Abnormal*, complete the following:

i. Describe any abnormalities _____

ii. Estimated effect on visual acuity? O None O 20/25- 20/40 O 20/50 – 20/100 O< 20/100 **(AnterChVAOD)**

1d. Iris neovascularization present: O Absent O Present, pupillary margin only
O Present, beyond the margin, but not in the angle O Present, in the angle **(IrisNeoCondOD)**

1e. Other anterior segment abnormality: O Yes O No **(OthAntSegAbnoOD)**
If *Yes*, complete the following:

i. Describe any abnormalities: _____

ii. Estimated effect on visual acuity? O None O 20/ 25- 20/40 O 20/50 – 20/100 O < 20/100
(OthAntSegAbnoEstVAOD)

ANTERIOR SEGMENT EXAM – LEFT EYE (OS)

Examination of lids/conjunctiva, cornea, iris, and anterior chamber

1. Anterior segment (other than lens): O Normal O Abnormal **(AntSegOthLensOS)**
If *Abnormal*, complete the following:

1a. Lids/Conjunctiva: O Normal O Abnormal **(LidsCondOS)**
If *Abnormal*, complete the following:

i. Describe any abnormalities _____

ii. Estimated effect on visual acuity? O None O 20/ 25- 20/40 O 20/50 – 20/100 O< 20/100 **(LidsEstVAOS)**

1b. Cornea (OD): O Normal O Abnormal **(CorneaCondOS)**
If *Abnormal*, complete the following:

i. Describe any abnormalities _____

ii. Estimated effect on visual acuity? O None O 20/25- 20/40 O 20/50 – 20/100 O< 20/100 **(CorneaEstVAOS)**

1c. Anterior chamber: O Normal O Abnormal **(AntChOS)**
If *Abnormal*, complete the following:

i. Describe any abnormalities _____

ii. Estimated effect on visual acuity? O None O 20/25- 20/40 O 20/50 – 20/100 O< 20/100 **(AnterChVAOS)**

1d. Iris neovascularization present: O Absent O Present, pupillary margin only
O Present, beyond the margin, but not in the angle O Present, in the angle **(IrisNeoCondOS)**

1e. Other anterior segment abnormality: ○ Yes ○ No **(OthAntSegAbnoOS)**
If *Yes*, complete the following:

i. Describe any abnormalities: _____

ii. Estimated effect on visual acuity? ○ None ○ 20/ 25- 20/40 ○ 20/50 – 20/100 ○ < 20/100
(OthAntSegAbnoEstVAOS)

To be eligible, the study eye cannot be aphakic or have substantial cataract that, in the opinion of the investigator, is likely to be decreasing visual acuity by 3 lines or more.

LENS ASSESSMENT – RIGHT EYE (OD)

1. Lens Status: ○ Phakic ○ AC IOL ○ PC IOL ○ Other IOL ○ Aphakic **(LensStatusOD)**

2. Lens (if phakic) or posterior capsule (if pseudophakic or aphakic) opacity:
○ None ○ Minimal, no significant effect on visual acuity ○ Visually significant **(LensOpacOD)**

If *minimal or visually significant*, complete the following:

2a. Describe any abnormalities _____

2b. Estimated effect on visual acuity (in the setting of an otherwise normal exam)?
○ None ○ 20/25- 20/40 ○ 20/50 – 20/100 ○ < 20/100 **(LensOpacVAOD)**

LENS ASSESSMENT – LEFT EYE (OS)

1. Lens Status: ○ Phakic ○ AC IOL ○ PC IOL ○ Other IOL ○ Aphakic **(LensStatusOS)**

2. Lens (if phakic) or posterior capsule (if pseudophakic or aphakic) opacity:
○ None ○ Minimal, no significant effect on visual acuity ○ Visually significant **(LensOpacOS)**

If *minimal or visually significant*, complete the following:

2a. Describe any abnormalities _____

2b. Estimated effect on visual acuity (in the setting of an otherwise normal exam)?
○ None ○ 20/25- 20/40 ○ 20/50 – 20/100 ○ < 20/100 **(LensOpacVAOS)**

POSTERIOR SEGMENT EXAM – RIGHT EYE (OD)

VITREOUS

1. Vitreous hemorrhage? ○ Yes ○ No ○ Cannot determine **(VitreousHemCondOD)**
If Yes, complete the following:

1a. Estimated effect on visual acuity? ○ None ○ 20/ 25- 20/40 ○ 20/50 – 20/100 ○ < 20/100
(VitreousHemVAOD)

2. Vitreous Haze
(*white blood cells based on view with 20D lens and indirect ophthalmoscope unless otherwise stated. Other media opacities, including vitreous hemorrhage, asteroid hyalosis or pigment should not be included in this grading or be graded*

as "6-cannot assess"): **(VitreousHazeOD)**

- O 0-Clear
- O 1-Trace haze; cells visible behind lens with slit lamp but 20D view clear (not red cells)
- O 2- Mild blur
- O 3- Vessels visible, but blurry
- O 4-No vessels visible, can see optic nerve
- O 5-Optic nerve not visible
- O 6- Cannot assess (media opacity or vitreous hemorrhage)

If vitreous haze present, complete the following:

2a. Estimated effect on visual acuity? O None O 20/ 25- 20/40 O 20/50 – 20/100 O < 20/100 **(VitreousHazeVAOD)**

DIABETIC RETINOPATHY

4-2-1 Rule
Severe NPDR is defined as having one or more of the following features present:
4- quadrants of hemorrhage
2- quadrants of venous abnormalities
1- intraretinal microvascular abnormalities (IRMA)

1. Diabetic Retinopathy Severity: (DRSeverityOD)

- O 0-None
- O 1-Microaneurysms only
- O 2-Mild/moderate NPDR
- O 3-Severe NPDR (4-2-1 rule)
- O 4-PDR (NVE, NVD and/or vitreous or preretinal hemorrhage considered due to DR) and/or prior scatter
- O 5-Cannot determine

If *4 (PDR and/or PRP present)* selected, complete the following:

1a. Are scars of scatter (panretinal) photocoagulation present? (ScarsPhotoPresOD)

- O No
- O Questionable or partial
- O Yes (at least 3 quadrants with scars ≤ 3 scar diameters apart)
- O Cannot assess (media opacity or vitreous hemorrhage)

1b. Are new vessels and/or vitreous/preretinal hemorrhage present? (VesVitHemPresOD)

- O No
- O Yes, post-PRP, not sufficient to justify additional PRP
- O Yes, post-PRP, additional PRP indicated
- O Yes, no prior PRP
- O Cannot assess (media opacity or vitreous hemorrhage)

To be eligible, the study eye must have definite retinal thickening due to diabetic macular edema involving the center of the macula.

2. DME on clinical exam: (DMEClinExamOD)

- O None
- O Present, center not involved
- O Present, center involvement uncertain
- O Present, center involved
- O Cannot determine

If *DME present*, complete the following:

2a. If DME is present, indicate how you would characterize it type – focal versus diffuse – in your own daily practice. You are free to use, or not use, OCT, FA, and/or fundus photos in addition to your clinical examination. (DMECharOD)

- O Typical focal (textbook example)
- O Predominately focal
- O Intermediate
- O Predominately diffuse
- O Typical diffuse (textbook example)

2b. Check the single most important examination used in reaching your answer. If two or three examination were about equally important, check those two or three.

☐ Clinical examination **(DMEClinExOD)**

☐ OCT **(DMEOCTOD)**

☐ FA **(DMEFAOD)**

☐ Red-free photographs **(DMERedFreePhotoOD)**

☐ Color photographs **(DMEColorPhotoOD)**

OTHER POSTERIOR SEGMENT ABNORMALITIES

1. Other posterior segment/fundus/optic disc abnormality:

O Yes O No O Cannot determine **(PostSegFundAbnoOD)**

If *Yes*, complete the following:

1a. Describe any abnormalities _____

1b. Estimated effect on visual acuity? O None O 20/ 25- 20/40 O 20/50 – 20/100 O < 20/100 **(PostSegFundAbnoVAOD)**

POSTERIOR SEGMENT EXAM – LEFT EYE (OS)

VITREOUS

1. Vitreous hemorrhage? O Yes O No O Cannot determine **(VitreousHemCondOS)**

If Yes, complete the following:

1a. Estimated effect on visual acuity? O None O 20/ 25- 20/40 O 20/50 – 20/100 O < 20/100 **(VitreousHemVAOS)**

2. Vitreous Haze

(*white blood cells based on view with 20D lens and indirect ophthalmoscope unless otherwise stated. Other media opacities, including vitreous hemorrhage, asteroid hyalosis or pigment should not be included in this grading or be graded as "6-cannot assess")*: **(VitreousHazeOS)**

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If vitreous haze present, complete the following:

2a. Estimated effect on visual acuity? O None O 20/ 25- 20/40 O 20/50 – 20/100 O < 20/100 **(VitreousHazeVAOS)**

DIABETIC RETINOPATHY

4-2-1 Rule
Severe NPDR is defined as having one or more of the following features present:
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To be eligible, the study eye must have definite retinal thickening due to diabetic macular edema involving the center of the macula.

2. DME on clinical exam: (DMEClinExamOS)

- O None
- O Present, center not involved

○ Present, center involvement uncertain
○ Present, center involved
○ Cannot determine

If *DME present*, complete the following:

2a. If DME is present, indicate how you would characterize it type – focal versus diffuse – in your own daily practice. You are free to use, or not use, OCT, FA, and/or fundus photos in addition to your clinical examination. (DMECharOS)
○ Typical focal (textbook example)
○ Predominately focal
○ Intermediate
○ Predominately diffuse
○ Typical diffuse (textbook example)

2b. Check the single most important examination used in reaching your answer. If two or three examination were about equally important, check those two or three.
☐ Clinical examination **(DMEClinExOS)**
☐ OCT **(DMEOCTOS)**
☐ FA **(DMEFAOS)**
☐ Red-free photographs **(DMERedFreePhotoOS)**
☐ Color photographs **(DMEColorPhotoOS)**

OTHER POSTERIOR SEGMENT ABNORMALITIES

1. Other posterior segment/fundus/optic disc abnormality:
○ Yes ○ No ○ Cannot determine **(PostSegFundAbnoOS)**

If *Yes*, complete the following:

1a. Describe any abnormalities _____

1b. Estimated effect on visual acuity? ○ None ○ 20/ 25- 20/40 ○ 20/50 – 20/100 ○ < 20/100 **(PostSegFundAbnoVAOS)**

COMMENTS